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SPECIAL ARTICLE

Failure of communication: a patient's story

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ABSTRACT

The first author, a patient who underwent elective caesarean section and felt pain necessitating conversion to general anaesthesia, describes the experience with particular reference to the perceived poor communication between her and her anaesthetist. This extended from the preoperative visit to the information provided to her general practitioner after discharge. She makes several suggestions which would have made her experience, and those of other patients in similar circumstances, less traumatic. The second author, who had no involvement in events and works in a different Trust, comments upon the events from the perspective of an obstetric anaesthetist.

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A patient's story

Introduction

My first son was born naturally. A slowly progressing labour meant I asked for an epidural but I then had my son with no intervention, despite his head circumference being on the 98th centile. During my second pregnancy I was diagnosed with an anterior placenta praevia and an elective caesarean section was booked. It was made clear that it would be a higher risk delivery and I might need a general anaesthetic.

Meeting my anaesthetist

We were in a ward beforehand and I had already changed into the operating gown. The consultant anaesthetist walked in looking down at my notes and, although he briefly looked up, he was brusque and made little eye contact. I don't remember it as a conversation, he was ticking things off.

In retrospect, I feel investing just a minute longer might have made me feel more of a connection. As it was, I shrugged off his indifference because I fundamentally believed in him. He was a consultant and, having

many friends who are medics, I knew how highly trained that meant he would be.

The best analogy I can give is that when you board a plane, you believe in the pilot's skill and you assume the pilot wants a safe landing as much you do. With an obstetric anaesthetist, a mother is quickly in an unusually intimate situation and a profound level of trust is given instantly. As a patient you have to accept it: you need the operation and you have to trust everyone will do their part competently – to doubt would cause an intolerable level of anxiety.

After the spinal injection

The anaesthetist was very confident it was in the right place. However, whilst the anaesthetic worked on my motor nerves, the effect was patchy with my sensory nerves. When asked whether I could feel the cold spray on my skin, I said that I could and the response was "Well, you may be able to feel the sensation of fluid on your skin, but can you feel it as cold?" I couldn't tell. I struggled to distinguish the difference between the sensation of the fluid and cold.

The anaesthetist waited and then repeated the test with the same question. This happened several times over what felt like 10 minutes.

I was very conscious of the room full of people waiting and I became embarrassed by the amount of time I seemed to be wasting. At the same time I felt stupid for being unable to answer what seemed such a simple question. I was aware of the expectation that the block

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should be working. I was lying on my back, looking up into the face of someone who was convinced that the block would be working. The anaesthetist was the expert and the person with authority. In my experience this was an immensely difficult situation in which to go against authority. Despite my real reservations because I was not sure, I decided the slight difference in sensation that I could feel must be sufficient. I gave the go ahead. I know I gave the wrong answer and I accept my responsibility in that.

Both accepting responsibility and feeling at fault was very tough. A year later, this was what I kept coming back to: how could I have allowed an operation to start when I was not sure? It was unthinkable and yet that was what I had done.

The relationship between patient and doctor

The paradigm of the doctor and patient working in equal partnership does not reflect the situation you have in an operating theatre. The patient is physically in a deeply subservient position and is not an equal in terms of skill and knowledge. Anaesthetists need to appreciate the extent to which their position, both metaphorically and physically, will influence the patient's thinking.

As suggested by the word 'theatre' clinicians are, in some respects, 'on stage'. It is necessary for the patient's peace of mind that their anaesthetist appears confident. Moreover, when an anaesthetist goes to place a spinal injection, they need confidence in their skills, because they need to believe they are going to get it right. But all this confidence can make it very difficult for a patient to express concern. The thing that led me was the anaesthetist's apparent conviction that all was well.

I feel that, after placing the spinal, the anaesthetist's manner needs to change such that it encourages the patient to speak up: they need to switch from being the 'technically adept wizard' placing the needle to a mode of 'patiently seeking approval' when testing the block. If you could safely assume that when you place a spinal it will work, you would not need to test at all. You test because you cannot afford to make that assumption. While the weight of your experience will be towards successful blocks, you cannot allow confirmation bias to creep in. It has to be a conscious effort to work on the basis that the block is not adequate until you can prove that it is.

What happened as the operation started

I was very aware that this was a higher risk caesarean section. As far as I was concerned, from the moment they started, I should not interrupt. I felt it was all time-critical: I did not know when they were going to cut through my placenta and I did not want to do anything that might result in harm to my baby. So it was a

significant act for me to ask to stop the operation and, each time I did, I apologised. My repeated apologies reflect the extent to which I felt I was being a nuisance.

The first incision felt like I was a bean bag being opened up and I vividly remember the sense of cleaving open. Things quickly got significantly worse.

I immediately said I could feel the surgery. I was offered Entonox to help with what the anaesthetist referred to as my "anxiety". I accepted but emphasised that I really could feel it. So the anaesthetist, who maintained his air of confidence and calm throughout, gave me alfentanil. He commented this would take "the edge off the discomfort". He explained that a general anaesthetic was an option if the pain became "too much". The wording used made me feel I should be able to cope better than I was. So I tried.

The operation was stopped a second time and I was given a second dose of alfentanil. I am not sure what was most effective – the accumulation of drugs or crushing my husband's hand and telling him not to break eye contact. However, I went in expecting to see my son born and I hung on for that. The pain was so much more than normal labour. It was uncontrollable, indescribable pain. Everything was confused by the sense that I should not be a distraction.

I heard my son's first cry, but did not see him before something happened that felt like a blunt instrument being run round my insides. I believe it may have been the surgeon running her hand round to remove my placenta. This was intolerable. I looked up at the anaesthetist who said "it's going to take another 20 minutes" and I asked for the general because I could not take any more.

I was reassured that my husband would hold the baby until I came round and I remember going under with a sense of enormous relief as feeling faded.

My next memory is of hearing a monitor beeping incredibly rapidly; I thought that did not sound good before realising it was my heart rate. When I opened my eyes, my husband was sitting where I could see him, with the baby in his arms.

Being offered the option of general anaesthesia

The anaesthetist was probably trying to give me an open choice. Choice in childbirth is often commented on and I would say that women should be able to express their preference on the available options. However, it is important to understand that lying open on an operating table, I was not in the best position to make the best decision for myself.

I have been told subsequently that if I felt the initial incision, it was a very poor block. It would have been more helpful if, instead of putting the onus on me to handle it, the anaesthetist had said "I am sorry this is happening but, if you can feel this, it is very likely to

get worse and in these situations we recommend switching to a general anaesthetic." I would not want anyone to feel abdominal surgery.

What happened afterwards

My initial reaction was a combination of shock and being pathetically grateful to the anaesthetist for rescuing me. He and one of his colleagues visited me on the three subsequent days I was in hospital. He stood as far as way from me as the room would allow. On one occasion, he commented "there is no accounting for science," while his colleague admitted that they had no idea why spinals sometimes did not work. Despite that verbal admission, nobody on the ward knew. I was on my own.

When I was discharged, my general practitioner received only the most basic information which stated I had had a routine caesarean section under local anaesthetic. When I mentioned the birth to the community midwife, she glanced at my notes, saw nothing and, clearly thinking I was making a fuss about nothing, she told me "never mind dear, the baby's alright, that's all that matters."

It was 10 months later that I went back to my general practitioner because I was having flashbacks and was finding it difficult to re-engage with life. It was only at this point that she realised I had undergone a general anaesthetic. She wrote to the hospital requesting an opportunity to go through my notes and received a letter back which stated I had been "conscious and comfortable" at the time of delivery. Had I been comfortable, there would have been no need for a general anaesthetic.

So my general practitioner wrote back. "Susanna does not want to make a formal complaint. She wants to offer constructive feedback so that future patients do not have a similar experience if such can be prevented." That letter was accompanied by an account by my husband. This elicited something of an apology from the anaesthetist and an invitation to discuss events with him. However, as my response to reading his letter was to be physically sick, I decided against it.

Meanwhile, the hospital offered an appointment to see the trauma counsellor. The time offered was three days before my son's first birthday and I felt that this came too late. By that stage, emotionally, I was not able to have those discussions because I could not face potentially making things worse. Moreover I felt any discussion would be limited by fear of legal recourse. I desperately needed things to be more positive.

Long-term impact

I had a life-saving procedure and my son was alright: I have a keen sense of perspective on that. However, I remember nothing of the first year of my son's life.

I never sought a diagnosis but my experience was consistent with the symptoms of post-traumatic stress disorder. For years I experienced flashbacks, irrational fears and wanting to avoid certain situations. I was so scared of going back into the same hospital that I delayed accepting medical intervention for my son's severe glue ear. I needed a guarantee from the ear, nose and throat consultant that we would not see the same anaesthetist. This seemed rational at the time. However, it now shocks me that I held off treatment which was so important developmentally. My son was moderately deaf from when he was born until grommets finally went in when he was 21 months old.

The impact was felt by the whole family. My sons had a mother who was physically present but in other ways absent because I was numb and disconnected. Owing to the fact that my notes did not reflect what happened, there was no support.

The aftermath

I realised I was not the only mother who had experienced difficulties. I was not in a position to do empirically robust research but I did a survey on-line, particularly focussing on the qualitative responses. One-hundred-and-fifty women gave details of their experiences. Very few had made formal complaints and yet there were heart-breaking descriptions. For example one mother had a termination rather than face the same situation again, and several more underwent sterilisation to prevent the possibility of further pregnancies.

To be entrusted with those stories meant I felt an imperative to communicate about the issues patients were experiencing. I wrote to the Presidents of the Royal College of Anaesthetists (RCOA) and Obstetric Anaesthetists' Association (OAA), opening up the opportunity for a measured discourse from which we could all learn.

Summing up

The messages I would like anaesthetists to take away fall into three areas:

1. Communication and testing

Firstly, nothing is 100%. You test because you cannot afford to make assumptions. The safest thing is to work on the basis that a block is not adequate until you can prove that it is. Secondly, I understand the need to be confident in your skills, but please reflect on the influence of perceived confidence and authority. Without saying a word, you can make someone agree with you – even against their better judgement. Thirdly, mother's overwhelming focus is on the safe delivery of her baby and, to that end, mothers will allow significant injury

to themselves. If a mother reports pain during surgery, believe her. If it becomes apparent that a block is not working, be clear about the likelihood that things will get worse. Apologise and make the recommendation to switch to general anaesthesia.

2. Duty of Candour

Duty of Candour legislation states the what and how – but it does not emphasise why.¹ I hope that sharing my experience may give insight into why candour is so important for the patient. Moreover, I would like to highlight an important difference between a doctor's ethical duty of candour and the statutory duty:^{1,2} the thresholds are different, being lower for the ethical duty while the threshold for statutory duty is higher. My concern is that the doctor's ethical duty remains even when an incident does not reach the statutory threshold. The apology is incredibly important to the patient: it needs to happen anyway. If you, as a clinician are unsure, perhaps the easiest rule is to consider what you would want if the patient were a close relative.

3. Communication with community care

Finally, your influence over the care a mother receives does not end when you stop seeing her. Because you can inform the next person in the system. If you don't pass on information, others will see nothing in the notes and will assume things were fine. So when the mother says "it was traumatic and I'm struggling" she'll get the response "well, the baby is alright, that's all that matters"...

To not be believed is to add insult to injury. Even a brief acknowledgement of difficulty and likely experience of trauma will go a long way toward ensuring support for mothers in these situations. It is the right thing to do. "Conscious and comfortable" is the aim and those words should never be a lie.

A final comment

I have always believed that no harm was intended and that mistakes offer the opportunity for learning. Initially, when I approached the RCOA and OAA, I was concerned about patients. It has been through having a dialogue with anaesthetists that I have come to understand how devastating it can be for clinicians when something goes wrong. My messages are intended to help protect anaesthetists as much as their patients. Through speaking for the OAA, I have met so many compassionate and conscientious professionals, who have overwhelmed me with their responsiveness and restored my faith in an entire profession. I would like to thank all those people.

An anaesthetist responds

Two points struck me forcibly when I heard Susanna's story. The first, and I hope that she will forgive me for this, is that, to an audience of obstetric anaesthetists, the sequence of events which she describes appears quite mundane. Most of us have encountered patients who have a spinal anaesthetic which is slow to spread, but which eventually reaches an apparently acceptable level, who then complain of pain during surgery, and who need conversion to general anaesthesia after delivery. We might remember these cases, reflect upon them and discuss them at our annual appraisal, but this is not the sort of rarity or catastrophic outcome that finds its way into morbidity and mortality meetings or case reports in journals.

Secondly, the anaesthetist in this case, while not covering himself in glory, does not appear to have performed at the sort of level which might attract professional or legal opprobrium. Susanna admits to having given him an optimistic assessment of the level of block before surgery started, he offered general anaesthesia when she felt pain, administered it when she gave the go-ahead, and visited her more than once postoperatively.

Despite this, Susanna has suffered considerable psychological trauma which she describes in unflinching detail, and which has, *inter alia*, impacted on her seeking medical treatment for her child. She has also told me that, like her correspondent, she would undergo a termination of pregnancy rather than face another caesarean section herself.

Susanna is not alone in having felt pain during caesarean section under neuraxial anaesthesia. In my own medicolegal practice, this is the commonest negligence claim made against obstetric anaesthetists, accounting for 21% of all such cases. Communication failure, the key factor in Susanna's story, is a feature in 48% of the 750 anaesthetic negligence claims which I have handled. We therefore have much to learn from Susanna.

It is too easy for us to forget how much impact an apparently minor complication can have on our patients, and how much influence we can have on outcome. This is reason enough to make sure that Susanna's story is heard by our specialty, but there are other lessons that she can teach us as well.

The most important of these is about communication. To us, an elective caesarean section under spinal anaesthesia in a healthy young woman is our bread and butter – we do it hundreds of times every year, and thousands of times during our working careers. To our patients, it is a frightening, often one-off, life-defining event. However routine to us, therefore, and however many other elective patients we have to assess when we arrive on the maternity unit in the morning,

we must work to establish personal contact and trust from the outset with each mother. The preoperative visit is key to this and, if it is true that 25% of job interviewers reach their decision whether to appoint someone within five minutes of the candidate entering the room,³ the same probably holds for a patient assessing their anaesthetist.

Susanna talks about being “on stage” in the operating theatre. I would reiterate this and say that it applies just as much – perhaps even more so – when we first approach a patient on the ward. Eye contact, a smile, a hand-shake, physically coming down to their level to talk to them, and knowing something about them in advance (their job, how many other children they have, their partner's name), are all small things that can make a profound difference.

Susanna then moves on to discuss communication in the operating theatre when testing the block. Her comments about the inequality of the doctor-patient relationship in these circumstances may well surprise those of us brought up in an era of shared decision-making and partnership, but a moment's thought is all that it takes to realise that what she says is right. When the patient is lying flat on the operating table surrounded by professionals in scrubs leaning over her and all eager to make a start, any concept of equality must fly out of the window. And, if an articulate professional like Susanna feels like this, how much more profound must it be for a less forceful individual? There is, of course, no easy answer to this. All we can do is to maximally empower the patient while ensuring that we and our colleagues do not appear overbearing. An anaesthetist of my acquaintance, since hearing Susanna's story, ensures that all in theatre are quiet and attentive, then clearly tells the patient that these people are happy to wait until she is ready, and that she is in charge of when the operation will start.

Susanna's observation about the confidence of the anaesthetist possibly acting as a two-edged sword, reassuring the patient but also inhibiting them from speaking out if things are not going according to expectation, is difficult but not impossible to reconcile with how we are taught to practice. She describes it as a shift from being a “technically adept wizard” to “patiently seeking approval”. If we can disconnect ourselves from the pervasive feeling that a poor block is an indication that we have failed our patient, then we should be able to confidently tell our patient that blocks do not always work effectively, that they will be the best judge of that, that we will test the block together, and that we are relying on them to tell us if it is good enough.

Current trends in provision of information and the consent process have led many to believe that it is no longer acceptable to recommend a course of action to our patients. This is not the case; it is not only acceptable but also good practice in many circumstances to

make a clear recommendation once the alternatives have been presented.⁴ Susanna enumerates the reasons why a patient in her situation will try to “muddle through” rather than opt for a general anaesthetic mid-caesarean section. We know better than our patients that, if they feel pain early in the procedure before uterine incision, this is only going to get worse as time progresses. Try simple analgesia like alfentanil by all means but, if this fails, do not be frightened to strongly suggest conversion. Coercion is not acceptable: recommendation is.

The need for good communication on the part of the anaesthetist extends into the postoperative period and to their interaction with other key personnel. The potential for psychological harm means that it is mandatory to make sure that those caring for the patient outside the hospital, particularly the community midwife and the general practitioner, are made aware of what has happened. This is a task that should not be delegated as this could lead to vital information being lost or distorted. Patients should be given the opportunity to return to discuss the events in question either with their own anaesthetist or if this is too painful, as was the case here, then with a sympathetic colleague. My impression from Susanna's account is that, even though she had such a traumatic experience during delivery, she would have suffered less psychological harm and would certainly have been far less angry and dissatisfied with her care if someone had only listened to her story, explained what had gone wrong, ensured that she was properly followed up and cared for when she got home and, of course, apologised. Her anaesthetist went some way to doing this, but not far enough.

As anaesthetists, we rightly pride ourselves on our technical skills. We can intubate the most inaccessible trachea, inject local anaesthetic around deeply-set nerves under ultrasound control, cannulate fine arteries and apply complex pharmacokinetic algorithms in real time. There were no technical failures in Susanna's case, but there were plenty of non-technical ones, and these centred squarely on poor communication. Good communication is a tricky skill to teach and to learn. It is difficult to define, to measure, and to research. But, like Supreme Court Justice Potter speaking in relation to obscenity, “We know it when we see it”.⁵ Thanks to Susanna's brave decision to share her experience and her trauma with us, I have every hope that we will see our communication skills, surely the true marker of the excellent obstetric anaesthetist, improve to the benefit of all of our patients.

References

1. Care Quality Commission. Regulation 20: Duty of Candour. Newcastle upon Tyne, 2015. http://www.cqc.org.uk/sites/default/files/20150327_duty_of_candour_guidance_final.pdf [accessed July 2016].

2. General Medical Council. Good Medical Practice: Domain 4, Maintaining Trust. London, 2013. http://www.gmc-uk.org/Good_medical_practice___English_1215.pdf_51527435.pdf [accessed July 2016].
3. Frieder RE, Van Iddekinge CH, Raymark PH. How quickly do interviewers reach decisions? An examination of interviewers' decision-making time across applicants. *J Occup Organ Psychol* 2016;**89**:48–223.
4. General Medical Council. Consent: Patients and doctors making decisions together. London, 2008.
5. Potter Stewart J. *Jacobellis v. Ohio*. 378 US 184 (1964).